

Practice Parameter for the Assessment and Treatment of Psychiatric Disorders in Children and Adolescents With Intellectual Disability (Intellectual Developmental Disorder)

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Intellectual disability (intellectual developmental disorder) (ID/IDD) is both a psychiatric disorder and a risk factor for co-occurring psychiatric disorders in children and adolescents. *DSM-5* introduced important changes in the conceptualization and diagnosis of ID/IDD, and current research studies clarify assessment and treatment of co-occurring psychiatric disorders in this population. Optimal assessment and treatment of psychiatric illness in children and adolescents with ID/IDD includes modifications in diagnostic and treatment techniques, appreciation of variations in the clinical presentation of psychiatric disorders, an understanding of the spectrum of etiologies of behavioral disturbance, and knowledge of psychosocial and medical interventions.

Key words: practice parameter, intellectual disability, child psychiatry, assessment, treatment

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The purpose of this Practice Parameter is to guide clinicians in the assessment and treatment of co-occurring psychiatric disorders in children and adolescents with intellectual disability (intellectual developmental disorder) (ID/IDD). This parameter updates the previous 1999 AACAP parameter¹ on the management of co-occurring disorders and incorporates new research findings and changes in the field. In particular, important changes in the definition and diagnosis of ID/IDD and in the field of genetics have occurred.

In the *DSM-5*, ID/IDD, unlike earlier classifications of mental retardation, is recognized as a psychiatric disorder. A goal of the *DSM-5* revision was to emphasize that the psychiatrist has a role in assessing intellectual deficits and in using the severity table to determine adaptive functioning. *DSM-5* shifts the focus away from the IQ number to emphasize adaptive reasoning and functioning and facilitates child and adolescent psychiatrists being actively involved in the diagnosis of ID/IDD, including etiological workup and specifiers of severity, as described in the diagnostic manual. An understanding of these procedures and processes assists clinicians working with this population to be most effective in treatment.

This parameter assumes familiarity with child development and the principles of child psychiatric diagnosis and treatment. As people with ID/IDD are a heterogeneous group with a wide range of disabilities and strengths, recommendations should be adapted to an individual's needs.

METHODOLOGY

This parameter is an update of the “Practice Parameters for the Assessment and Treatment of Children, Adolescents and Adults with Mental Retardation and Comorbid Mental Disorders,” which was based upon a review of literature published prior to 1999. The current Practice Parameter is based on a comprehensive search of publications produced since 1999. Articles were retrieved from PubMed, PsychInfo, Cochrane, and CENAHl databases. Searches were inclusive and used both MeSH headings and keywords.

PubMed was searched using the MeSH Major Topic intellectual disability. The initial search yielded 69,413 articles. This search was limited to English, humans, “all child (0 to 18 years)” and the years from January 1, 1999, to January 21, 2019. In addition, this retrieval was limited to the following: classical article, clinical trial, comparative study, controlled clinical trial, evaluation studies, guideline,